

TRICARE INDIVIDUAL PROVIDER CONTRACT

PARTIES

Regence BlueShield ("SHAREHOLDER"), a Washington nonprofit corporation, and
James Fred Raymond, MD ("Provider").

EFFECTIVE DATE

This contract (the "Agreement") shall be effective (the "Effective Date") on the first day of the month following verification and approval of Provider's credentials by SHAREHOLDER and TriWest Healthcare Alliance Corp. ("TriWest"). The Effective Date of this Agreement is 6/1/06
(to be inserted by TriWest after verification and approval of Provider's credentials).

RECITALS

- A. SHAREHOLDER has subcontracted with TriWest to establish a provider network for TriWest in conjunction with TriWest's contract with the Department of Defense to provide managed health care services to TRICARE Beneficiaries.
- B. Provider is a health care provider licensed by and in good standing with the State of Washington, and desires to participate in SHAREHOLDER's network for TRICARE beneficiaries.

Therefore, the Parties agree as follows:

AGREEMENT

I. DEFINITIONS

- A. Active Duty - Full-time duty in the Uniformed Services of the United States. It includes duty on the active list, full-time training duty, annual training duty, and attendance while in the active Military Service, at a school designated as a Service school by law or by the Secretary of the Military Department concerned.
- B. Authorization - Approval for requested services, procedures or admission that is obtained prior to services being rendered.
- C. Clean Claim - A claim which does not require a pre-existing condition investigation, coordination of benefits information, accident information and/or subrogation information, medical records or any other information to adjudicate the claim.
- D. Copayments - Deductibles, copayments and/or cost sharing amounts payable by a TRICARE beneficiary, as set forth in the TRICARE Provider Guide.
- E. Covered Services - Services for which benefits are available to TRICARE beneficiaries in accordance with the rules, regulations, policies and instructions of the TRICARE Management Activity.

- F. Emergency Care - TRICARE considers an emergency medical condition a condition manifesting itself by "acute symptoms of sufficient severity, including severe pain, such that a prudent layperson could reasonably expect the absence of medical attention to result in placing the beneficiary's health in serious jeopardy, serious impairment to bodily function, or serious dysfunction of any bodily organ or part."
- G. Health Care Coordinators (HCC) - Medical professionals employed by TriWest to assist in finding medical care for TRICARE beneficiaries in the military or civilian system. Health Care Coordinators are responsible for facilitating referrals/authorizations.
- H. Hospital-based Physician - TMA defines Hospital-based Providers as professional providers (except Interns, Residents, and Fellows), either directly employed or under contract to a hospital who provide services under a hospital owned Tax Identification Number for either inpatient or outpatient care, where the hospital is not acting as a billing agent for the provider.
- I. Institution - a general, acute-care hospital, a specialty hospital such as a behavioral health hospital, or a skilled nursing facility.
- J. Medically Necessary - The appropriate and necessary treatment of the patient's illness or injury according to accepted standards of medical practice and TRICARE policy.
- K. MTF - Military Treatment Facility.
- L. Network Provider - A provider who has contracted to render covered services to TRICARE beneficiaries.
- M. Primary Care Manager (PCM) - A provider who is designated as a Primary Care Manager and is selected by a TRICARE Prime beneficiary or assigned by an MTF commander to provide primary care services. PCM's may be an individual, group, or MTF. PCMs include: Internists, Family Practitioners, Pediatricians, General Practitioners, Obstetricians/Gynecologists, Physician Assistants, Nurse Practitioners, or Certified Nurse Midwives.
- N. Reimbursement Rates - The rates set forth in Exhibit I.
- O. RFP - The Department of Defense's Request for Proposal No. MDA906-02-R-0006, as amended, for the Managed Care Support Services under the TRICARE Program. Definitions included in the RFP are incorporated herein by reference, except as otherwise provided.
- P. Tail Insurance - When a provider has professional liability insurance on a claims-made basis (as opposed to an occurrence-based policy), tail insurance provides coverage after the termination of the claims-made insurance policy for losses resulting from claims that are filed after the expiration of the claims-made insurance policy.
- Q. TRICARE (formerly CHAMPUS) - A program established to provide health coverage for family members of active duty military personnel, for retirees and their families, and for other TRICARE beneficiaries.
- R. TRICARE Provider Guide - Manual of operational policies and protocols for TRICARE that will be furnished to Provider.



- S. TRICARE beneficiary - Any person eligible to receive covered services under the rules, regulations, policies and instructions of TMA. TRICARE beneficiaries may include, but are not limited to, active duty members of the Armed Forces of the United States of America and their dependents, retired members of the Armed Forces of the United States of America and their dependents, non-Department of Defense Uniformed Services (the Public Health Service, the United States Coast Guard, and the National Oceanic & Atmospheric Administration) and certain North Atlantic Treaty Organization beneficiaries.
- T. TRICARE/Champus Maximum Allowable Charge (CMAC) - The reimbursement methodology for services rendered to TRICARE beneficiaries. CMAC is updated by the federal government on an annual basis.
- U. TRICARE Extra - The Preferred Provider option (PPO) under the TRICARE Program in which benefits are provided through Network Providers reducing the TRICARE beneficiary's cost share.
- V. TRICARE Management Activity (TMA) - formerly known as OCHAMPUS and TRICARE Support Office - Area within Department of Defense responsible for overseeing the TRICARE Program.
- W. TRICARE Prime - A Health Maintenance Organization (HMO)-type option under the TRICARE Program in which TRICARE beneficiaries enroll and receive enhanced primary and preventive benefits at a reduced cost share. Medical care is coordinated by a PCM.
- X. TRICARE Standard - Standard benefits available under Champus. Similar to a traditional indemnity plan, with an annual deductible, cost-share, and annual out-of-pocket catastrophic cap.
- Y. TriWest - TriWest Healthcare Alliance Corp. and, as applicable, its subcontractors.

II. TERM; TERMINATION

- A. Term - This Agreement shall commence upon the Effective Date and continue for an initial two year term. Thereafter, both Parties agree that the term of this Agreement shall automatically be extended for one year periods until TriWest no longer has a contract with TRICARE Management Activity (TMA) or unless terminated by either Party as permitted in the Agreement.
- B. Termination without Cause - Either Party may terminate this Agreement without cause upon at least ninety (90) days prior written notice to the other Party.
- C. Immediate Termination - SHAREHOLDER shall have the right to immediately terminate this Agreement upon written notice to Provider upon the occurrence of any of the following events:
 - 1. Provider's Washington license or another state or federal license or authorization to do business is reduced, restricted, suspended, or terminated (either voluntarily or involuntarily), or Provider's other applicable license or accreditation necessary to perform any services contemplated by this Agreement is reduced, restricted, suspended, or terminated (either voluntarily or involuntarily); or
 - 2. Provider's professional liability coverage as required under this Agreement is reduced below required amounts or is no longer in effect; or

3. Provider fails to meet SHAREHOLDER's or TriWest's recredentialing, quality management or utilization management criteria, or fails to comply with quality management or utilization management processes; or
 4. Provider fails to provide material information or provides erroneous information on Provider's credentialing application or recredentialing application.
- D. Material Breach - Either Party may terminate this Agreement for any material breach of this Agreement by the other Party, but only if that breach is not cured within thirty (30) days after written notice to the breaching Party.
- E. After termination of this Agreement, Provider shall use reasonable efforts to notify any TRICARE beneficiaries that Provider is no longer a TRICARE provider when TRICARE beneficiaries seek care from Provider. In addition, Provider shall cooperate with SHAREHOLDER to ensure a smooth transition for TRICARE beneficiaries from Provider to another network provider.
- F. Termination of this Agreement shall not relieve either Party of any obligation to the other Party in accordance with the terms of this Agreement with respect to services furnished prior to such termination and shall not relieve Provider of the obligation to cooperate with TriWest in arranging for the transfer of care of TRICARE beneficiaries then receiving treatment.

III. PROVIDER'S RESPONSIBILITIES

- A. Provider agrees to treat TRICARE beneficiaries according to the terms and conditions of this Agreement. Provider shall accept the Reimbursement Rates (less the amount of any copayments payable by the TRICARE beneficiary) as the only payment expected from TriWest and TRICARE beneficiaries for covered services, and for all services paid for by the TRICARE Program. TRICARE beneficiaries are responsible only for copayments. The Reimbursement Rates shall apply to active duty and civilian claims, to enrollees and to non-enrollees, and to all TRICARE beneficiaries whose care is reimbursed by the Department of Defense, regardless of their residence. In no event will Provider be paid for such services more than the TRICARE/Champus Maximum Allowable Charge (CMAC) or what is permissible under federal law or TRICARE policy.
- B. Provider shall collect applicable copayments from TRICARE beneficiaries. The copayments depend upon the sponsor's grade and military status. Except as otherwise provided in this Paragraph B., Provider may not bill TRICARE beneficiaries or active duty personnel for any service that is non-covered or disallowed. Provider shall not routinely waive copayments. Except for copayments, Provider agrees that in no event (including, but not limited to, nonpayment or breach of this Agreement by TriWest or TriWest's insolvency) shall Provider bill or collect for covered services from a TRICARE beneficiary or active duty member, and this provision shall survive termination of this Agreement. Provider shall not require payment from a TRICARE beneficiary for any excluded or excludable service that the TRICARE beneficiary received unless the TRICARE beneficiary has been properly informed that the services are excludable and has agreed in advance of receiving the services, in writing, to pay for such services. A TRICARE beneficiary who is informed that care is potentially excludable and proceeds with receiving the potentially excludable service shall not, by receiving such care, be construed to have entered into an agreement to pay. Provider acknowledges that payment shall not be allowed for a non-covered service unless the TRICARE beneficiary or Active Duty member is properly informed and agrees in a separate writing. Any waivers must be specific as to the details of the excluded or non-covered service. General agreements to pay, such as those signed by the TRICARE beneficiary at the time of service, are not evidence that the TRICARE beneficiary knew specific services were excluded or excludable or that the TRICARE beneficiary agreed to pay.



- C. Provider shall submit claims for Covered Services on behalf of TRICARE beneficiaries and active duty personnel. Provider shall use best efforts to submit claims within thirty (30) days after the provision of the services. All claims will be submitted electronically. No payment shall be made for a claim submitted more than one (1) year after the provision of the covered service.
- D. Provider shall participate in Medicare (accept assignment) and submit claims on behalf of all TRICARE and Medicare beneficiaries.
- E. Provider agrees to being reported to the Department of Veterans Affairs (DVA) as a TRICARE network provider. DVA shall have the right to directly contact Provider and request that he/she provide care to Veteran Administration (VA) patients on a case by case basis. Provider is never obligated to see the VA patient, however if seen, any documentation of the care rendered to the VA patient and reimbursement for the care is a matter between the referring VA Medical Center (VAMC) and Provider. The referral and instructions for seeking reimbursement from the VAMC will be provided by the patient at the time of the appointment. If Provider has initialed "Yes" Section A. of Exhibit 3. Provider has agreed to accept requests from the DVA to provide care to veterans.
- F. Provider agrees to being reported to the Civilian Health and Medical Program of the Veteran's Administration (CHAMPVA) as a TRICARE network provider. If Provider has initialed "Yes" Section B. of Exhibit 3, Provider agrees to accept assignment for CHAMPVA beneficiaries.
- G. Provider shall comply with all policies and procedures set forth in the TRICARE Provider Guide, including without limitation credentialing, peer review, referrals, utilization review/management and quality assurance programs and procedures established by TriWest or TRICARE including submission of information concerning Provider and compliance with pre-authorization requirements, care approvals, concurrent reviews, retrospective reviews, discharge planning for inpatient admissions and prior authorization of referrals. Such requirements may concern active duty personnel as well as TRICARE beneficiaries.
- H. Provider shall provide and maintain policies of general and professional liability (malpractice) insurance to insure Provider against any claim for damages arising by reason of personal injury or death resulting directly or indirectly from the performance of this Agreement. Such insurance shall be subject to the approval of SHAREHOLDER, but shall not be less than one million dollars (\$1,000,000) per claim and three million dollars (\$3,000,000) in the aggregate per year for each professional performing services. Provider shall provide SHAREHOLDER with a certificate of such insurance upon execution of this Agreement, entitling SHAREHOLDER to receive thirty (30) days' prior notice of any change in coverage or termination or expiration of coverage. If the insurance is on a claims-made basis, Provider shall obtain tail insurance satisfactory to SHAREHOLDER upon any termination of coverage and containing an extended reporting endorsement for a period of not less than three (3) years after the termination of this Agreement.
- I. If Provider offers behavioral health services, and the TRICARE Beneficiary authorizes release of the information, Provider shall submit, to the TRICARE Beneficiary's PCM, a copy of the record of the treatment provided.
- J. Provider or designee shall attend an initial educational seminar (and periodic update seminars) in order to obtain an understanding of the requirements of TRICARE.
- K. Provider's facilities shall be accessible to handicapped individuals as required by applicable federal and state and/or local laws and regulations.



- L. Provider shall comply with federal and any applicable state laws and regulations concerning the confidentiality and security of the medical records of TRICARE beneficiaries
- M. As soon as it is aware of them, Provider must notify SHAREHOLDER of any actions, policies, determinations or internal or external developments that may have a direct impact on Provider's ability to perform its obligations under this Agreement.
- Actions requiring notifications by Provider include, but are not limited to:
1. Any change in ownership, specialty services provided or location of facility(s);
 2. Action against or lapse of Provider's license, certification, accreditation or certificate of authority;
 3. Loss of hospital privileges; and
 4. Reduction in insurance coverage below the required limits or termination of insurance coverage.
- N. Providers shall participate in and cooperate with TriWest's Case Management and/or Utilization Management Plans described in the TRICARE Provider Guide, and developed in accordance with the policies, rules, and regulations of TriWest and TRICARE.
- O. Provider agrees to provide a TRICARE beneficiary with a copy of his or her medical record at no charge, to include a narrative summary and other documentation of care within two (2) business days of the request when the TRICARE beneficiary or TRICARE beneficiary's guardian presents (i) a copy of official change of station orders or orders changing the TRICARE beneficiary's or sponsor's status from active duty to retired or separated from service and (ii) a properly executed medical release.
- P. Provider agrees to provide copies of medical records to TriWest within two (2) business days of TriWest's request, to permit TriWest to conduct peer review, quality assurance and utilization review. At the Provider's request TriWest will reimburse Provider for the cost of photocopying and postage. The reimbursement rate for photocopies is currently set at \$.07 per page.
- Q. Provider acknowledges and agrees that the National Quality Monitoring Contractor (NQMC) shall be responsible for reviewing requests from TRICARE beneficiaries and Provider for an appeal or reconsideration of certain denials of coverage by TriWest and Provider will cooperate with the NQMC in such appeal process. Provider agrees the NQMC may release all review data obtained by the NQMC to TriWest.
- R. Provider understands and agrees that all Covered Services provided for TRICARE Prime enrollees, except emergency services, outpatient mental health services, and services provided under a Point of Service option, must be referred from the PCM to a Network Provider or an MTF provider, and authorized by the applicable Health Care Coordinator or other designee of TriWest.
- S. Provider agrees that TriWest and its designee shall have access, upon demand and at reasonable times, to the books, records and papers of Provider relating to the health care services provided to TRICARE beneficiaries and active duty personnel, to the costs thereof, and to copayments received by Provider from TRICARE beneficiaries for covered services. TriWest and its designee shall have the right to inspect, at reasonable times, Provider's facilities upon five (5) days' prior notice to Provider. Provider will provide adequate space to TriWest and its designee for the conduct of on-site inspections and reviews and shall cooperate in the conduct of such review activity. Provider will photocopy and deliver to TriWest or its designee all information required for off-site review by TriWest of Provider's performance under this Agreement within thirty (30) days of a request by TriWest. This section shall survive termination of this Agreement.



- T. If Provider is designated as a specialist, the wait time for visits shall not exceed four (4) weeks. If Provider is designated as a Primary Care Manager, non-emergent, routine visits shall be scheduled within one (1) week and urgent visits within twenty-four (24) hours. The wait time in the Provider's office for non-emergencies shall not exceed thirty (30) minutes from the scheduled appointment time. Provider shall provide typed consultation or referral reports, operative reports, and discharge summaries to the TRICARE beneficiary's initiating provider within ten (10) working days of the encounter. In urgent/emergent situations, a preliminary report of a consultation shall be conveyed to the TRICARE beneficiary's initiating provider within 24 hours (unless best medical practices dictate less time is required for a preliminary report) by telephone, fax or other means.
- U. Provider acknowledges and understands that the MTF has a first right of refusal to provide medical services to TRICARE Prime beneficiaries who are referred for any services by their PCM.
- V. Provider acknowledges that any hospital who serves TRICARE Beneficiaries and at which Provider has admitting privileges will require Provider when he/she is the attending physician to have a signed and dated acknowledgment that Provider has received the following notice:

"Notice to Physicians: TRICARE payment to hospitals is based in part on each patient's principal and secondary diagnoses and the major procedures performed on the patient, as attested to by the patient's attending physician by virtue of his or her signature in the medical record. Anyone who misrepresents, falsifies, or conceals essential information required for payment of federal funds may be subject to fines, imprisonment or civil penalty under applicable federal laws."

The acknowledgment shall be completed by Provider at the time that Provider is granted admitting privileges at the hospital, or before or at the time Provider admits his or her first patient. Existing acknowledgments signed by physicians already on staff remain in effect as long as the physician has admitting privileges at the hospital.

- W. Provider shall comply with additional responsibilities, if any, set forth in Exhibit 2.
- X. Provider acknowledges that this is not an exclusive agreement, and that neither TriWest nor SHAREHOLDER represents, warrants, or guarantees that Provider will be utilized at all by any TRICARE Beneficiary, active duty member or by any number of TRICARE beneficiaries or active duty personnel.
- Y. Provider agrees that Provider will not discriminate in providing Covered Services under this Agreement against any TRICARE beneficiary or Active Duty member on the basis of his/her enrollment or non-enrollment in TRICARE Prime, source of payment, sex, age, race, color, religion, national origin, health status, or disability.
- Z. Provider shall comply with all final HIPAA ASC X 12N Transaction standards as promulgated by the Secretary, Department of Health and Human Services for implementation effective October 2003.

IV. ADDITIONAL RESPONSIBILITIES APPLICABLE TO PCM PROVIDERS

- A. By initialing and completing Exhibit 4, Provider agrees to serve as a PCM and to accept the maximum number of beneficiaries as set forth in said Exhibit. As a PCM, Provider shall be available by telephone or by appointment twenty-four (24) hours per day, seven (7) days per week to ensure timely evaluation of a patient's health needs. If the Provider is unavailable, it is the responsibility of



the Provider to arrange for coverage by a TRICARE certified provider. A covering physician shall be a contracted provider or a provider who accepts assignment (i.e. agrees to accept the TRICARE-determined allowable amount as payment in full for covered services and supplies provided to the TRICARE beneficiary) and shall be subject to TRICARE policies, rules, and regulations, including without limitation credentialing and certification requirements.

The wait time for visits to the PCM shall not exceed the following:

1. Routine visits - one (1) week;
2. Urgent care - twenty-four (24) hours; and
3. Well visits - four (4) weeks.

In a non-emergency, the wait time in Provider's office shall not exceed thirty (30) minutes from the scheduled appointment time.

- B. As a PCM, Provider agrees to provide or arrange to provide, the preventive services required under the TRICARE Prime program. All referrals by a PCM shall be in accordance with the TRICARE Provider Guide, and the policies, rules and regulations of TriWest and TRICARE in effect from time to time.

V. TRIWEST'S RESPONSIBILITIES

TriWest will periodically include Provider's name, gender, location, whether the Provider is accepting new patients, status (if any) in the Reserve or National Guard, and specialty in a directory of Network Providers. Provider is responsible for notifying SHAREHOLDER of any changes of address or specialty within ten (10) business days.

VI. PAYMENT TO PROVIDER

- A. TriWest will use best efforts to process clean claims within thirty (30) days of receipt and will make payment directly to Provider for covered services rendered by Provider to TRICARE beneficiaries or active duty personnel in accordance with the Reimbursement Rates set forth in Exhibit I. Provider understands and agrees that SHAREHOLDER is not the insurer, payer, guarantor or underwriter of the payment of benefits to Provider and agrees that SHAREHOLDER shall not be responsible for payment of any claims submitted by Provider for Covered Services provided to TRICARE beneficiaries.
- B. TriWest may deny payment for services or supplies deemed by TriWest to be not medically necessary or at an inappropriate level under definitions and determinations of TRICARE. TriWest will utilize a standard industry code review system in adjudicating claims and determining appropriate levels of coding.
- C. Payments to Provider may be disallowed or reduced for noncompliance with required utilization review/management programs and procedures, including without limitation failure to obtain a required pre-authorization or continued stay approval even if the services are not denied on necessity or appropriateness grounds. In the case of failure to obtain a required pre-authorization or continued stay approval, the reduction shall be at least ten percent (10%) of the Reimbursement Rate as set forth in Exhibit I. Amounts that are disallowed or reduced may not be billed to TRICARE beneficiaries or active duty personnel.



- D. Provider understands and agrees that there may be payment adjustments through the remittance or return of underpayments, overpayments, and adjustments for retroactive terminations or denials of coverage.

VII. GENERAL PROVISIONS

A. Amendment

SHAREHOLDER may modify this Agreement by providing Provider with sixty (60) days prior written notice. Provider shall have thirty (30) days after receipt of such notice to object in writing to a proposed amendment. If Provider objects timely and in writing to the proposed amendment, SHAREHOLDER may terminate this Agreement upon thirty (30) days' prior written notice to Provider. If Provider fails to object timely and in writing to a proposed amendment, Provider shall be deemed to have accepted such proposed amendment.

Except as specified above and unless specified elsewhere in this Agreement or allowed/prohibited by applicable law, this Agreement may be amended or modified only in writing signed by the authorized personnel of each Party.

B. Applicable Law; Jurisdiction; Venue

This Agreement is governed by the laws of the State of Washington and applicable federal law. The Parties consent to the jurisdiction of and to venue for any dispute involving this Agreement in the state courts of the State of Washington or the United States District Court for the District of Washington.

C. Assignment

Except as permitted in this Agreement, neither Party may assign or transfer any right, benefit, obligation or duty under the terms of this Agreement to any third party without the prior written consent of the other Party.

D. Authority

Each person signing this Agreement certifies that he/she has the appropriate authority to bind the respective Party.

E. SHAREHOLDER's Relationship to the Blue Cross and Blue Shield Association

Provider expressly acknowledges his/her its understanding that this Agreement constitutes an agreement between Provider and SHAREHOLDER, that SHAREHOLDER is an independent corporation operating under a license from the Blue Cross and Blue Shield Association (the "Association"), an association of independent Blue Cross and Blue Shield Plans, permitting SHAREHOLDER to use the Blue Shield Service Mark in the State of Washington, and that SHAREHOLDER is not contracting as the agent of the Association.

Provider further acknowledges and agrees that he/she/it has not entered into this Agreement based upon representations by any person, entity or organization other than SHAREHOLDER and that no person, entity or organization other than SHAREHOLDER shall be held accountable or liable to Provider for any of SHAREHOLDER's obligations to Provider created under this Agreement. This Paragraph shall not

create any additional obligations whatsoever on the part of SHAREHOLDER other than those obligations created under other provisions of this Agreement.

F. Compliance

Provider warrants and certifies that he/she/it is in compliance with all local and federal laws applicable to provider's business of providing health care services, including but not limited to, the provisions of the Americans with Disabilities Act and the Health Insurance Portability and Accountability act as they may apply to Provider.

G. Coordination of Benefits/Third-Party Liability

Provider agrees to make inquiries of TRICARE beneficiaries regarding other health insurance coverage. If there is another entity providing coverage for the TRICARE beneficiary, Provider shall bill that entity first and provide information regarding that carrier to TriWest when it submits the claim to TriWest for covered services provided to the TRICARE beneficiaries. TRICARE coverage shall always be secondary, except when TRICARE beneficiaries have coverage with Indian Health Services or Medicaid. Provider further agrees to cooperate in subrogation, Workers' Compensation and other third-party recovery programs to the extent permitted or required by applicable law.

H. Dispute Resolution

1. If Provider believes that TriWest incorrectly denied all or part of a claim and desires to obtain a review of the benefit determination, Provider shall, within ninety (90) days of initial determination:
 - a. submit a written request for review to TriWest. ATTN: Provider Services; and
 - b. Include in the written request the items of concern regarding TriWest's determination and all additional information (including medical information) supporting Provider's belief that the denial was incorrect.

On the basis of the information supplied with the request for review, together with any other information available to it, TriWest will review its prior determination. Provider will be notified in writing of TriWest's decision and the reasons for the determination within sixty (60) days of TriWest's receipt of the request for review.

If Provider still believes that TriWest's determination of payment or non-payment is incorrect and/or has information that was not previously available for review when submitted to TriWest, Provider may direct a second request for review in writing to TriWest within sixty (60) days of receipt of the prior determination. The TriWest reviewer will follow the procedures outlined in the TRICARE Provider Guide for processing second reviews.

2. In the event that any claim or controversy arising out of or relating to this Agreement, or any claimed breach thereof, cannot be resolved by the Parties as provided in Section VII Paragraph H. Subsection 1 above or in the normal course of business, each Party shall designate a member of its senior management to meet in an attempt to resolve the dispute.



A dispute that cannot be resolved to the satisfaction of the Parties in this manner shall be referred for binding arbitration in accordance with the commercial dispute arbitration rules of the American Arbitration Association or such other rules as may be agreed to by the Parties.

Judgment upon an award in arbitration may be entered in any court of competent jurisdiction, or application may be made to such court for a judicial acceptance of the award and enforcement, as the law of the state having jurisdiction may require or allow.

I. Entire Agreement

This Agreement, including referenced Exhibits, contains the entire understanding of the Parties and supersedes all prior agreements between the Parties with respect to the same subject matter.

J. Mutual Indemnification

Provider shall hold harmless and indemnify SHAREHOLDER and TriWest for, from, and against any Provider-related claims, losses, damages, liabilities, costs, expenses or obligations arising out of or resulting from Provider's wrongful or negligent conduct in the performance of this Agreement including, but not limited to, the provision of health care services by Provider. SHAREHOLDER shall hold harmless and indemnify Provider for, from, and against any losses, damages, liabilities, costs, expenses or obligations arising out of or resulting from SHAREHOLDER's wrongful or negligent conduct in the performance of this Agreement.

K. Notice

All notices and other communications to a Party must be in writing, hand delivered, delivered by prepaid commercial courier service with tracking capabilities, faxed, or delivered by the U.S. mail to the address listed on the signature page. The Parties may change the address of record by notifying the other Party of the new address. Notice shall be complete upon the earlier of actual receipt or five (5) days after being deposited into the U.S. mail. Notices and other communications in writing need not be mailed either by registered or certified mail, although a signed return receipt received through the U.S. Post Office shall be conclusive proof as between the Parties of delivery of any notice or communication and of the date of such delivery.

L. Paragraph Headings

The paragraph headings used in this Agreement have been inserted for convenience of reference only and do not in any way modify or restrict the meaning of any of the terms or provisions of this Agreement.

M. Regulatory Requirements

Those portions of the Federal Acquisition Regulations (the "FAR," 48 C.F.R. Ch. 1.) and the Department of Defense Supplement to the FAR (the "DFARS," 48 C.F.R. Ch. 2.) mentioned in Section I of the RFP are incorporated into this Agreement with the same force and effect as if they were given in full text, and for such purposes, Provider shall be deemed the "Contractor" or "Offeror." Upon request, their full text will be made available.

N. Relationship of the Parties

The relationship of the Parties is not and shall not be construed or interpreted to be a partnership, joint venture or agency. The relationship between the Parties is an independent contractor relationship.

O. Release

PROVIDER ACKNOWLEDGES THAT A NUMBER OF FUNCTIONS UNDER THIS AGREEMENT WILL BE PERFORMED BY TRIWEST AS SET FORTH IN THIS AGREEMENT, INCLUDING, BUT NOT LIMITED TO, CLAIMS PAYMENT AND UTILIZATION REVIEW. PROVIDER AGREES TO THIS DELEGATION OF FUNCTIONS TO TRIWEST AND FURTHER AGREES THAT SHAREHOLDER SHALL NOT BE LIABLE FOR PAYMENTS UNDER THIS AGREEMENT OR FOR NEGLIGENT OR INTENTIONAL WRONGDOING OR BREACH OF THIS AGREEMENT BY TRIWEST. TRIWEST SHALL BE SOLELY LIABLE FOR ITS ACTIONS AND INACTIONS AND FOR ALL PAYMENTS DUE TO PROVIDER UNDER THIS AGREEMENT.

P. Third-Party Beneficiary

TriWest shall be a third-party beneficiary of this Agreement and shall be entitled to enforce Provider's obligations under this Agreement, and Provider shall be entitled to enforce TriWest's obligations under this Agreement.

Q. Trade Name Ownership

The Parties acknowledge that SHAREHOLDER has the sole right to use, in Washington, the "Blue Shield" trade name and service mark.

R. Waiver

There shall be no waiver of any term, provision or condition of this Agreement unless in writing and signed by both Parties.

S. Severability

If any provision of this Agreement is deemed illegal, unenforceable or in conflict with any law of a federal, state or local government having jurisdiction over this Agreement, the validity of the remaining sections shall not be affected. In addition, the illegal, unenforceable or invalid provision shall be replaced by a mutually acceptable provision, which, being valid, legal and enforceable, comes closest to the intention of the parties concerning the illegal, unenforceable or invalid provision.

T. Survivability

The obligations of Sections III. A., III. B., III. C., III. H., and VII.J shall survive the termination of this Agreement.

Signatures appear on next page.

Intending to be legally bound, the Parties have executed this Agreement as of its Effective Date.

SHAREHOLDER

Regence BlueShield, a Washington
Nonprofit Corporation

By: *Audrey J. Nudd*
Name: Audrey J. Nudd
Title: Vice President, Provider Services

Date: 02/22/06

Whose main address is:
1800 Ninth Avenue
Seattle, WA 98101

Fax # (206) 626-6145

Accepted by TriWest:

TriWest Healthcare Alliance Corp.

By: *Lisa D. Stevens*
Signature

Lisa D. Stevens
Vice President, Provider Services

Whose main address is:
P.O. Box 42049
Phoenix, AZ 98580

Fax # (602) 564-2456

Provider

James Fred Raymond, MD
Name

Radiation Oncology
Specialty

912018691
Tax ID

By: *James Fred Raymond*
Signature

Whose main address is:
4525 3rd Ave NE SE Ste 100
~~Olympia, WA 985065065~~ Lacey, WA. 98503-1010

Fax # (360) 412-8974

Exhibit 1

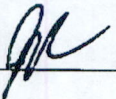
Physician Reimbursement Rates

PROVIDER NAME: James Fred Raymond, MD

TIN: 912018691

Provider agrees to accept the lesser of a zero percent (0 %) discount off the TRICARE/Champus Maximum Allowable Charge (CMAC) or a twenty percent (20%) discount off Provider's billed charge as the Reimbursement Rate. Provider acknowledges that, as set forth in Section III Paragraph A. of the Agreement, this is the exclusive reimbursement he/she will receive for the provision of covered services except for applicable co-payments.

In no event will Provider be paid more than what is permissible under federal law or TRICARE policy.

 (Provider initial)

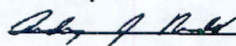
 (SHAREHOLDER initial)



Exhibit 2

Additional Responsibilities

There are no additional responsibilities at this time.

Exhibit 3

**Participation in Department of Veterans Affairs and
Civilian Health and Medical Program of the Veteran's Administration**

- A. Provider agrees to accept requests from the Department of Veterans Affairs to provide care to veterans.

Yes ☒ SM (initial) No ☐ _____ (initial)

- B. Provider agrees to accept assignment for Civilian Health and Medical Program of the Veteran's Administration (CHAMPVA) claims.

Yes ☒ SM (initial) No ☐ _____ (initial)